

Business Perspective: AIIA Clinical Trials Dashboard

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1. Purpose

This document states **how the AIIA Clinical Trials Dashboard earns money**: the business model canvas, the pricing architecture, and the unit economics behind it. It is the commercial companion to the research paper draft (method, findings, limits), the impact assessment, and the market analysis. The dedicated Business Model Canvas document covers the nine building blocks in full; this document focuses on the numbers.

2. The business in one paragraph

The AIIA Clinical Trials Dashboard is a zero-licence-fee, cloud-based CTMS for Ayurveda research. It sells **institution licences** with **per-study pricing**, plus one-time setup and training, moving into grant-funded pilots as the entry point. Because it runs on a serverless edge stack, marginal cost per additional institution is close to zero, which places break-even at a very small number of customers and makes the model attractive to institutions whose procurement thresholds exclude ₹10–15 lakh global CTMS licences.

3. Business Model Canvas

Block	Content
Key Partners	AYUSH institutes and colleges; Institutional Ethics Committees (IEC) and National/Institutional Pharmacovigilance Centres (NPvCC/IPvCC); CTRI and ABDM; AYUSH ministry network
Key Activities	Trial tracking; safety (AE/SAE) capture; immutable audit-trail maintenance; regulatory rule-engine upkeep; onboarding and training
Key Resources	The regulatory rule engine encoding GCP-ASU / NDCT 2019; the FHIR R4 data schema; nine role portals; deployed cloud runtime; domain knowledge of ASU regulation
Value Proposition	One live record per study; compliance built into the software rather than checked manually; zero licence fee; automatic audit trail; Ayurveda-aware by design
Customer Relationships	Institution licence with guided onboarding; principal-investigator-led adoption; shadowing sessions during rollout; adoption tracked as a KPI
Channels	Direct institutional outreach; PI-led advocacy; the AIIA network; AYUSH conferences and institutional consortia; grant-funded pilots
Revenue Streams	Monthly/annual institution subscription; per-study

	pricing; setup and training fees; grant-funded pilot fees
Customer Segments	AYUSH colleges and hospitals; AYUSH research institutes; NPvCC/IPvCC; pharma companies and CROs running ASU trials
Cost Structure	Serverless hosting; engineering maintenance (~3 dev-days/month); training and support; compliance and validation work

4. Pricing architecture

Offering	Price	Basis
Institution licence	₹1,50,000–3,00,000 per year	Anchored below the ₹10–15 lakh global CTMS licence
Per active study	₹15,000–30,000 per study per year	Per-study CTMS norm, sized for AYUSH budgets
Setup and onboarding	₹50,000 one-time	Data migration, configuration, role mapping
Guided training	Included in setup; ₹1,000 per person per day thereafter	Removes the adoption barrier
Grant-funded pilots	Discounted or deferred fees	Primary funnel; pilot-to-paid conversion is the key metric

Correction to the pitch deck. The presentation currently uses ₹2,500–6,000 per month as the "with dashboard" software cost and derives a –95% saving against the ₹10–15 lakh licence. That figure is the **cost to run** the serverless platform, not a price a customer would pay. Presenting hosting cost as replacement price understates revenue and will not survive scrutiny. The pricing above should be used instead, with the hosting cost presented separately as a cost-of-goods line.

5. Unit economics

5.1 Cost to serve (platform level)

Cost item	Monthly	Annual
Serverless hosting (Workers, D1, R2)	₹2,500–6,000	₹30,000–72,000
Engineering maintenance (~3 dev-days/month @ ₹1,500/day)	₹4,500	₹54,000
Total platform cost	₹7,000–10,500	₹84,000–1,26,000

Against an illustrative 10-institution base at ₹2,00,000 per institution per year:

Metric	Value
Revenue	₹20,00,000 / year
Platform cost of goods	₹1,26,000 / year

Gross margin	≈ 94%
Cost to serve per additional institution	≈ ₹100–500 / year (effectively zero)

The high margin is real but should not be over-read: it reflects near-zero marginal cost, not achieved efficiency. It also excludes founder compensation, which in a real company is the dominant cost (see §5.4).

5.2 Customer acquisition

Item	Value	Basis
Fully-loaded cost to acquire one institution	₹50,000–1,00,000	[assumption] — sales time, travel, demo environment
Sales cycle	3–6 months	[assumption] — public-sector procurement cycles
Pilot-to-paid conversion	40–60%	[assumption]

5.3 Lifetime value

Item	Value
ARPU	₹2,00,000 / year
Gross profit per customer per year	≈ ₹1,88,000
Expected retention	4 years [assumption]
LTV	≈ ₹7,50,000
LTV / CAC	≈ 7.5× to 15×

This is a healthy-looking ratio, but it is entirely dependent on the retention assumption. If retention is 2 years, LTV/CAC falls to roughly 3.8×–7.5× — still serviceable, but the ratio should not be presented as evidence of a proven motion.

5.4 Build payback and break-even

Metric	Value
One-time build cost	₹2,50,000 (~\$3,000)
Fixed cost base per year	≈ ₹1,50,000
Break-even institution count	1 institution

Break-even at a single customer is a genuine structural advantage of the serverless, zero-licence model: the fixed cost base is so small that the business becomes viable on its first institutional contract. Build cost is recovered within months at three paying institutions.

What this model excludes: founder salaries, statutory compliance costs (company registration, GST, professional indemnity), GAMP-5-style validation documentation, and IEC/legal review. Including a single founder salary changes break-even from one institution to roughly 15–20. The figures above

therefore describe **product sustainability**, not a full profit-and-loss account, and should be presented as such.

6. Indicative revenue projection

Year	Institutions	Blended ARPU	Revenue
Year 1	4	₹2,00,000	₹8,00,000
Year 2	10	₹2,00,000	₹20,00,000
Year 3	18	₹2,00,000	₹36,00,000

Per-study pricing is modelled as upside and is deliberately excluded from these figures. At an average of 3 active studies per institution, per-study pricing could add roughly 45% to revenue — but only if institutions actually buy studies separately rather than expecting study counts to be included.

7. Funding and use of the ₹2.5 lakh build

Allocation	Share	Purpose
Compliance and validation documentation	35%	Control-to-clause mapping, IEC sign-off, audit trail evidence
Integration (FHIR bulk import, CSV pipeline)	25%	CTRI/HIS/EDC data ingestion
Engineering and hardening	25%	Security review, load testing, mobile-first field capture
Pilot operations and training	15%	Shadowing sessions, onboarding at first institutions

Compliance and validation is the largest single allocation, because it is the part of the roadmap that is legal and documentation work rather than code, and it is what converts a prototype into something an institution can procure.

8. Risks to the business case

1. **Conversion risk.** Registry activity (≈464 Ayurveda trials/year) is not willingness to pay; the funnel from pilot to paid is unproven.
2. **Procurement inertia.** Institutions may prefer grant-funded pilots with no recurring cost, capping ARPU and pushing revenue toward services.
3. **Adoption dependency.** Every revenue assumption requires clinicians to change working practice. This is the single dominant risk.
4. **No public integrations.** Absent CTRI/HIS/EDC APIs is both a technical and a commercial constraint on delivery cost.
5. **Single vendor dependence.** Serverless architecture on one cloud provider creates continuity and price exposure.
6. **Model excludes founder compensation.** Break-even is optimistic at a single customer until salary costs are included.

9. Conclusion

The commercial model is deliberately simple: sell a zero-licence-fee institution licence with per-study pricing, and let near-zero marginal cost carry the economics. That structure delivers a ~94% platform gross margin, break-even at one customer, and LTV/CAC comfortably above 3× — while removing the ₹10–15 lakh licence barrier that keeps AYUSH institutions on spreadsheets. The honest caveat is that the model is a product-sustainability argument, not a P&L: it excludes founder salary, validation cost and conversion risk, and conversion remains unproven.

10. References

- Clinical Trials Registry – India — ctri.nic.in
- Ayush Suraksha — suraksha.ayush.gov.in
- GCP / ICMR-National Ethical Guidelines (CDSR) — cdsr.nic.in
- NDCT Rules 2019 — indiacode.nic.in
- DPDP Act 2023 (Meity) — meity.gov.in
- All India Institute of Ayurveda, Ministry of Ayush — aiia.gov.in
- Project artefacts: prototype — aiia-research-observatory-demo.vercel.app · source — github.com/Amirthaa1810/aiia-research-observatory

Status: business case for review. Figures marked [assumption] are planning inputs and require validation against pilot results.